



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : احلام حسن العبد شعبان

العنوان : ش الفاتح العريش ثان
شمال سيناء

الحي / المدينة : العريش ثان

المحافظة : شمال سيناء

تاريخ تسجيل الإستمارة : 27-09-2026

رقم التليفون : 01203556910

رقم البطاقة : 25503093400069

العمر (تاريخ الميلاد) : 09-03-1955 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض
بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: احلام حسن العيد شيمان National ID: 25503093400069	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



د. م. م.
موجهة لعميد سبتين تحت الجبل





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : **خلال حسن العبد شرفان**

Sex:

MDT form

3- Medical number:
Female

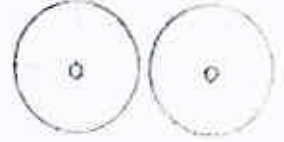
B3226141

Old Med :

2720-2024

2-10 number: 25503093400069

Patient Age : 70 Y



Family history

Family history
-ve

Phone

01261035817

BMI

18.5

REASON OF HGT

marital status

Marital status

NRS Score

NRS Score

GROUP

GROUP

Date of HGT

2025-04-20

Co morbidity

Co Morbidity
dm

Menopausal
status

Menopausal status
Menopausal

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

It breast malignant mass

Mets Workup :

CT CAP :free

BX
IDC

ER
8/8

(8 cores)

PR
8/8

(8 cores)

Her 2 Neu
3

(2 cores)

SISH

(1 core)

Ki67
19

T
2

N
0

M
0

Sonomammography

PATIENT HISTORY

pt received 15 wks TC +wk 16,17 taxol only 25/2/2025

received one cycle phesgo

LT SM+SLNB: residual IDC GII

free deep margin, nipple, areola LN 0/4, PT0N0

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

CONTINUE PHESGO UP TO 17 CYCLE + FEMARA + ZOMETA Q 6 MONTH

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

Patient Name : الأستاذة / احلام حسن العبد شعبان	Request No. : 13000987
Patient No. : 25503093400069	Request Date : 19/03/2025
Path No. : 1083/25	Verification Date : 07/04/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 69 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data:

Left breast cancer post NACT.

Nature of the specimen:

Left simple mastectomy sentinel lymph node biopsy specimens collected at 19/3/2025.

Gross Examination:

Left simple mastectomy specimen measured 18x11x3.5 cm with covering skin ellipse measured 15x8 cm.

- Tumor extent: A clipped greyish white firm tumor bed site
- Size: 2.5x1.5x1 cm.
- Skin Involvement: Nipple & areola were unremarkable grossly.
- Distance from margins: 0.2 cm away from inked deep margin.
- Left sentinel lymph node biopsy specimen: A fibrofatty tissue piece measured 3x3x0.5 cm entangling four lymph nodes up to 1x0.5x0.5 cm.
- Frozen section:
- Free SLNs (0/3).

Microscopic examination:

- Histological type: Residual invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II (2+2+2).
- Actual tumor cellularity : 20%.
- Therapy effect in form of fibrosis and hyalinosis.
- Ductal carcinoma in situ: Not detected.
- Skin involvement: Free nipple and areola.
- Lymph node involvement: Free (0/4), one more node is discovered in paraffin examination.
- Resection Margins: Free deep margin.
- Histologic response to therapy: Pathologic partial response.

Verified by :

dr.esraa magdy

Printing Date : 13/04/2025 13:16:37

Page 1 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأمثلة / احلام حسن العبد شعبان	Request No. : 13000987
Patient No. : 25503093400069	Request Date : 19/03/2025
Path No. : 1083/25	Verification Date : 07/04/2025
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 69 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Diagnosis:

Left breast cancer post NACT, simple mastectomy and sentinel lymph node biopsy specimen :

- Residual invasive duct carcinoma grade II.
- Actual tumor cellularity: 20%.
- Free deep margin.
- Skin: Free nipple and areola.
- Free lymph nodes: (0/4).
- Pathologic partial response.
- Stage: ypT2, ypN0.
- RCB: 1.762, RCB class II.

Dr. Marwa Mahmoud
Dr. Esraa Magdy

Verified by :
dr.esraa magdy

Printing Date : 13/04/2025 13:16:37

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corridor - Misr El Qadima - Cairo, Egypt



Patient Name :	الأستاذة / أسماء حسن السيد شعير	Request No :	13002640
Patient No. :	25503093400069	Request Date :	18/08/2024
Path No. :	SR-1170/24	Verification Date :	27/08/2024
Sex :	Female	Financial Category :	تغطية التأمين - تأمين الدولة
Age :	69 Y	Department :	Out Patient
Referring Doc :		Registered by :	marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.
Nature of Specimen: Slide Revision

Gross Examination:

- 1 paraffin block and 2 slides labeled by (patient's name in arabic) were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation + nuclear pleomorphism + mitosis): Grade II (3+2+1)
- * DCIS: Focal DCIS.
- * Lymphovascular emboli: Positive.
- * Perineural invasion: No.

Diagnosis:

Left breast mass, Slide revision:
Invasive duct carcinoma G II with focal DCIS and positive lymphovascular emboli.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

Printing Date : 68/09/2024 08:33:03

Page 1 of 1

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قاعة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - معمر الشاذلي - معمر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Msr El Qadima - Cairo, Egypt



Patient Name : الأستاذة / منال محمد شحات
Patient No. : 25503093400069
Path No. : SR-1170/24
Sex : Female
Age : 69 Y
Referring Doc. :

Request No. : 13002640
Request Date : 18/08/2024
Verification Date : 01/09/2024
Financial Category : فئة المرضى - Out Patient
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Slide Revision

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9)

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8)
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8)
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+)
- Ki67: 19% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Slide revision:

Invasive duct carcinoma G II with focal DCIS and positive lymphovascular emboli.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 19% of tumor cells.

Scoring system: "ER, PR"

Verified by :

DR.Menna Nabil

Printing Date : 08/09/2024 08:33:07

Page 1 of 2

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Patient Name	المرضى / اسم المريض	Request No.	13002640
Patient No.	25503093400069	Request Date	18/08/2024
Path No.	SR-1170/24	Verification Date	01/09/2024
Sex	Female	Financial Category	أورام - فئة الدولة
Age	69 Y	Department	Out Patient
Referring Doc.		Registered by	marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: 'Her2'

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
DR Menna Nabil

Printing Date : 08/09/2024 08:33:07

Page 1 of 2

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Hera No. (4) - First Floor - Building No. 387 - Nile Corniche - Mit El Qadima - Cairo, Egypt



Ahlan Hassan El-Abd Shaban

Female

Q42574

9/30/2024 11:45:38 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of left breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- Abnormal osseous tracer uptake is seen at proximal shaft of left forearm bones that would account for bone metastasis.
- Focal intense abnormal tracer uptake is noted at right knee?? Likely at upper end of right tibia either osteoarthritic changes or true metastasis.
- Degenerative changes of the spine are noted.

Opinion

- Two worrying osseous lesions as described above for radiological correlation.

Dr. Mohamed AlaaEldin

Prof. Ahmed Kandeel, MD



Patient Name: Ahlam Hassan Al Abd Shaaban	Date : 18/08/2024
Exam : C.T ABDOMEN & PELVIS	Age : 69 Year(s)
Medical ID: 2730/2024	Pat. ID: 77701

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of breast cancer.

I Chest:

- Left breast LOQ parenchymal soft tissue lesion is seen hardly abutting underlying chest wall muscles measures about 1.5×1.9 cm along its axial dimensions. Subcentimetric ipsilateral axillary (level I & II) lymph nodes.
- Enlarged right axillary level I and II lymph nodes, the largest at level I measures 1.4×2.2 cm in maximum axial diameters.
- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitory lesions.
- Subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.
- Small sliding hiatus hernia is noted.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenuation. No sizable focal lesions on non-contrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Calcular gall bladder.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- Reported case of breast cancer revealed:
- Left breast mass lesion and contralateral right axillary lymph nodes for SM correlation.
- No sizable pulmonary, hepatic or osseous deposits on non-contrast basis.

Dr. Ali Elsisy, MSc.

Signature-Prof. Menna-Talleh Hanafy

26/08/2024 04:15



Patient Name: Ahlam Hassan Al Abd Shaaban	Date : 18/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 70 Year(s)
Medical ID: 2720/2024	Pat. ID: 77701

bilateral breast ultrasound

Indication:

left breast pathgologically proven malignant mass on NACT for assessment.

Technique

- Bilateral breast ultrasound

Report:

- Regressive course as regards the left breast LOQ irregular spiculated isoechoic mass lesion, seen at 4 o'clock 2cm from the nipple, 0.6cm from the overlying skin, measuring 18x10 mm (was 20x17 mm, in prior study)
- No right breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LN, bilaterally
- Free left infra and supra-clavicular regions.

Opinion:

- Partial radiological response of the left breast LOQ malignant mass (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. BASMA MOHMAED

18/02/2025 11:44 AM



SCAN & LABS

مركز النيل للأشعة والتحاليل

Name : Ahlam Hassan Al-Abd Shaaban	Date : 14-09-2026 09:47
Branch : Dokki	Medical No. : 461203
Ref.Physician Prof : Goustav Roussey Egypt	تعليق مستشفى : Ins.Company :

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 71-year-old female with left breast carcinoma (status post-surgical resection, chemotherapy, current hormonal therapy) presenting with diffuse bony aches. Evaluated for skeletal metastases.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Homogeneous symmetric physiological tracer distribution throughout the axial and appendicular skeleton, including the skull, spine, ribs, sternum, pelvis, and extremities.
- No focal areas of abnormal increased or decreased uptake suggestive of osteoblastic or osteolytic metastases.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

- Negative whole-body bone scan: No scintigraphic evidence of osseous metastatic disease.
- Correlate clinically and follow standard oncology surveillance protocols.

Dr. Alaa Nagi

MD Nuclear Medicine

N I L E D I A G N O S T I C C E N T E R

Name : Ahlam Hassan Al-Abd Shaaban

Date : 14-09-2026 09:47

Branch : Dokki

Medical No. : 461203

Ref.Physician Prof : Goustav Roussey Egypt

تعلق مستشفى : Ins.Company :

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Dr. Alaa Nagi

MD Nuclear Medicine

N I L E D I A G N O S T I C C E N T E R